QUICK REFERENCE

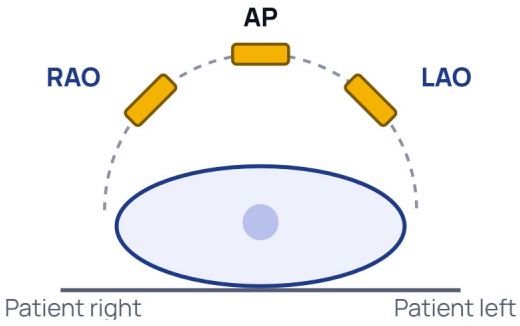
Cath Lab Views Cheat Sheet

Coronary angiography projections at a glance.

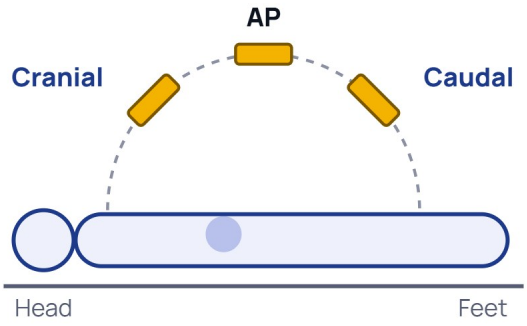
Dr. RRP CARDIO

Learn. Understand. Apply.

How to name a view



Seen from the patient's feet



Side view

- **RAO / LAO:** the detector swings to the patient's right or left.
- **Written as:** "LAO 40, caudal 30" (side first, then tilt).
- **Cranial / caudal:** the detector tilts toward the head or the feet.
- **Two views:** confirm every lesion in two views about 90° apart.

Left coronary: best views

Target	Best views	Why
Left main and bifurcation	Spider (LAO caudal), AP caudal, RAO caudal	Opens the LAD and LCx origins
Proximal LAD	LAO cranial, RAO caudal, spider	Origin with less overlap
Mid LAD and diagonals	AP cranial, RAO cranial, LAO cranial	Spreads diagonals away from the LAD
Septals	RAO cranial, LAO cranial	Septal branches run down from the LAD
Distal LAD and apex	Lateral, RAO cranial	Follows the LAD to the apex
Proximal LCx and ramus	Spider, RAO caudal, AP caudal	Circumflex origin and ostium
OMs and distal LCx	RAO caudal, AP caudal, LAO caudal	Marginals clear of the LAD

Typical starting angles adjust to the patient

AP cranial Cranial 30–40	**AP caudal** Caudal 20–30	**RAO cranial** RAO 30, cranial 20–30	**RAO caudal** RAO 30, caudal 20–30
LAO cranial LAO 30–50, cranial 20–30	**Spider (LAO caudal)** LAO 40–60, caudal 20–40	**Lateral** LAO 90	**Heart position** Angles shift with habitus

Educational use only. Not patient-specific medical advice.

© 2026 Dr. RRP Cardio · rrpcardio.com

Right coronary and reference

Right coronary: best views

Target	Best views
Ostium and proximal RCA	LAO 30–45, LAO cranial
Mid RCA	LAO 30–45, RAO 30, lateral
Distal RCA and crux (PDA, PLV)	LAO cranial, AP cranial

LAO is the workhorse for the RCA. Add cranial tilt to split the PDA from the posterolateral branches.

Other common views

Study	View
LV gram	RAO 30: anterolateral, apical and inferior walls. LAO 45–60: septum and lateral wall.
Aortic root	LAO 30–40

Coronary dominance

Right dominant (about 85%)	RCA gives the PDA
Left dominant (about 8%)	LCx gives the PDA
Co-dominant	Both supply the inferior wall

TIMI flow grades

Grade	What you see
0	No antegrade flow beyond the occlusion
1	Contrast penetrates but does not fill the distal vessel
2	Distal vessel fills, but slowly
3	Normal, brisk filling

Significant stenosis

Epicardial vessel	70% or more
Left main	50% or more
FFR	0.80 or lower
iFR	0.89 or lower

Pitfalls to avoid

- **Foreshortening:** a segment looks short. Change the angle by 20–30°.
- **Overlap:** a branch hides the lesion. Pick a view that spreads them.
- **Ostial lesions:** catheter damping or deep intubation can mislead. Disengage and re-inject.
- **Spasm:** consider intracoronary nitrate before calling a lesion.
- **One view only:** always add a second view about 90° away.
- **Centre the lesion:** pan the table and collimate to cut dose.

Memory aid Cranial for the LAD. Caudal for the circumflex. Spider for the left main. LAO for the RCA.

Educational use only. Angles are typical starting points: adjust to the patient and operator preference.

© 2026 Dr. RRP Cardio · rrpcardio.com